

Your VA Passes the IRS Test. Does It Pass Rhode Island's?

Why Massachusetts and Rhode Island Use a Second, Stricter Test — and Why Behavioral Health Practices Are Especially Exposed

A Practical Guide for Medical & Behavioral Health Practices in RI and MA

What We'll Cover

01 Why passing the federal test isn't the whole story in RI or MA

02 The ABC test, prong by prong

03 The prong that quietly fails most healthcare VAs

04 Why behavioral health practices carry extra exposure

05 What's actually at stake in each state

06 What helps your position, and your next steps



Passing One Test Doesn't Mean You Pass Both

Most guidance on 1099 vs. W-2 — including a lot of what's on Reddit and Google — only covers the federal IRS test. In Massachusetts and Rhode Island, that's only half the picture.

FEDERAL (IRS)

Common-law test weighing behavioral control, financial control, and relationship type together. No single factor decides it.

Governs federal payroll tax treatment.

STATE (MA & RI)

A strict, three-part ABC test. Every worker is presumed an employee until the employer proves all three parts — not just the total picture.

Governs state wage law, unemployment insurance, and workers' comp — independently of the IRS.

The ABC Test, Prong by Prong

Unlike the federal test, all three prongs must be satisfied — fail one, and the worker is an employee by default.



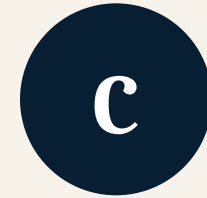
Free From Control

Is the worker free from your direction over how, when, and where the work gets done — both on paper and in actual practice?



Outside Your Usual Business

Is the work outside the usual course of your business, rather than a core part of how your practice actually operates?



An Independent Trade

Does the worker have their own independently established business, offering the same services to others in the market?



Prong A: Free From Control

This mirrors the federal behavioral-control factor — the most familiar part of the analysis for most practice owners.

Weighs against passing this prong:

- You set specific hours or require a fixed schedule
- You provide step-by-step instructions on how tasks are done
- You supervise the work directly, day to day
- The worker uses your systems, logins, and templates exclusively

Supports passing this prong:

- Worker sets their own hours and workflow
- You describe the outcome; they determine the method
- No direct daily supervision of their process
- Worker uses their own tools, systems, or subcontractors

Note: This prong alone is rarely where healthcare VAs fail — Prong B, next, is usually the real obstacle.

B

Prong B: The One That Trips Up Healthcare

The worker's service must be **outside the usual course of your business**. For a behavioral health or medical practice, that's a narrow needle to thread.

Ask yourself:

- Would my practice function without this role being filled by someone, in some form?
- Is this task part of the operational engine of patient care (scheduling, intake, billing, insurance follow-up) rather than a one-off side project?

The hard truth: scheduling, intake coordination, insurance verification, and billing support are not incidental to a medical or behavioral health practice — they're part of how the practice runs every single day. That makes Prong B genuinely difficult to satisfy for a VA doing this work, regardless of how independent the arrangement otherwise looks.



Prong C: An Independently Established Trade

The worker needs a genuine, independently established business of the same nature — not just a side gig that happens to be you.

Weighs against passing this prong:

- You are their only client, and have been for months
- They don't market VA services to any other practices
- No business registration, website, or other client contracts exist
- The arrangement started as, and remains, an informal handshake

Supports passing this prong:

- Worker actively serves multiple practices concurrently
- They operate under a registered business name or entity
- They carry their own liability insurance and set their own rates
- A real, functioning business existed before this engagement

Note: A single-practice VA who's been with you a year rarely clears this bar either — the exclusivity itself is evidence against it.



Why Behavioral Health Practices Are Exposed

VA support has been aggressively marketed to solo and small therapy practices as a low-overhead alternative to hiring front-desk staff — the exact role a receptionist or office manager traditionally filled

Intake, scheduling, insurance verification, and billing are core operational tasks in behavioral health — not peripheral ones, which is precisely what Prong B tests for

HIPAA-compliant VA vendors often provide the VA with practice-branded email addresses and direct EHR access, reinforcing integration into daily operations

Solo practitioners often engage one VA exclusively for 10–20+ hours a week, which cuts against genuine independent-business status under Prong C

A Common Scenario

“
I run a solo therapy practice in Providence. My VA handles scheduling, insurance verification, and intake forms, about 15 hours a week. She's 1099 and invoices me monthly. My accountant says the IRS test looks fine. Are we actually okay?

Running it through Rhode Island's ABC test:

Prong A — Control	Possibly passes, if she truly sets her own hours and process
Prong B — Usual Course of Business	Very likely fails — scheduling, intake, and insurance verification are core to how a therapy practice runs day to day
Prong C — Independent Trade	Likely fails — 15 hrs/week for one practice, with no indication of other clients or a registered business

The federal test being “fine” says nothing about Rhode Island exposure. Two out of three ABC prongs failing means employee status by default under state law.



What's at Stake in Massachusetts

Civil liability

Misclassified workers can recover unpaid wages, overtime, and the value of lost benefits — potentially at treble (3x) damages

Criminal exposure

Willful misclassification is a criminal offense: up to \$25,000 and a year in jail for a first offense, up to \$50,000 and two years for repeat violations

State contract bar

Employers found in violation can be barred from contracting with the Commonwealth of Massachusetts

Multi-agency reach

The Attorney General's Fair Labor Division, Department of Unemployment Assistance, and Department of Industrial Accidents can each pursue this independently



What's at Stake in Rhode Island

Full burden on the employer

The worker is presumed an employee from the start; the practice must affirmatively prove all three ABC prongs

Civil penalties

The RI Department of Labor and Training can pursue civil penalties once a misclassification complaint is investigated

Criminal referral

The Attorney General's Office can bring criminal charges where misclassification is established

Independent state agencies

Unemployment insurance and workers' compensation exposure are assessed separately from any federal resolution

Yes — and It Happens Constantly

A worker can be a legitimate federal 1099 contractor and still be a state-law employee in RI or MA — at the same time, for the same role.



Passes Federal
Passes State

Cleanest position — genuinely independent by both standards



Passes Federal
Fails State

The trap: federally fine, but a state-law employee for wage, unemployment, and workers' comp purposes



Fails Federal
Passes State

Rare in practice — state tests are generally stricter than federal



Fails Both

Highest exposure across every agency at once



Does This Still Apply If Your VA Is Remote?

Many VAs supporting RI/MA practices work from elsewhere entirely — another state, or overseas. That doesn't automatically remove state exposure.

Where the business operates

A practice based in RI or MA is generally still subject to that state's wage, unemployment, and workers' comp laws for work it directs and controls — regardless of where the worker sits

Where the worker performs the work

The worker's own state (or country) may separately impose its own classification, tax, and labor rules on the same relationship

Overseas contractors

Foreign-based VAs generally fall outside U.S. state wage law, but this doesn't resolve federal tax reporting questions, and platform/agency arrangements add another layer

The practical takeaway

“My VA doesn't live in Rhode Island” is not, by itself, a reason to assume Rhode Island law doesn't apply to how your practice engages her



What Actually Helps Your Position

If you're genuinely confident a VA relationship is independent, these strengthen (but don't guarantee) your position:



Evidence the VA actively serves other practices — not just references, but visible concurrent work



A registered business name, its own website or listing, and its own liability insurance



A written agreement describing outcomes and deliverables, not hours or supervised tasks



Billing at a project or deliverable rate, not an hourly rate resembling a wage



No practice-branded email address, and no mandatory use of internal systems where avoidable

Reality check: For most solo-practice VA arrangements handling core operational tasks, Prong B remains genuinely difficult to satisfy — documentation helps at the margins, but doesn't change the nature of the work itself.



Your Next Steps



List every 1099 worker supporting your practice's day-to-day operations



Run each one through all three ABC prongs — not just the federal test



Flag anyone performing core operational work (scheduling, intake, billing) for a closer look



Gather evidence of independent business status for anyone you believe genuinely qualifies



Talk to a professional before your next filing deadline, not after a claim or audit arrives

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